



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : زينب محمد امين محمد
المحافظة : الجيزة
الحى / المدينة : اطفح
العنوان : 0
رقم البطاقة : 27302142100582 رقم التليفون : 01156631358
العمر (تاريخ الميلاد) : 14-02-1973 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 07-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: زينب محمد امين محمد National ID: 27302142100582	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
 وزارة الداخلية
 بطاقة تعريف شخصية

زينب
 محمد أمين محمد
 قرية القليبات
 مركز اطلق - الجيزة

٢٧٣٠٢١٤٢١٠٠٥٨٢

KN2469019

٢٠٢٤/٠٤/٠٤
 ٧٣٠٢١٠٠٥٨٢
 ربة منزل
 انش
 عليه كامل محمد حسن
 المنطقة سارية حق ٢١/٠٤/٢٠٢٤

١



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : زينة محمد أمين محمد

Sex:

3- Medical number:

Female

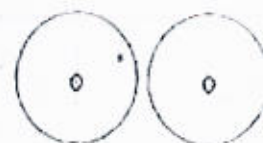
83209832

Old Med:

1622/2024

2-ID number: 27302142100582

Patient Age: 52 Y



Family history

Familial history
+ve

Phone

Phone

01156631358

BMI

24

REASON OF MDT

REASON OF MDT
Justification

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

2025-04-17

Co morbidity

Co morbidity
+ve

Menopausal
status

Menopausal status
Postmenopausal

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

LT UOQ ill-defined irregular mass lesion with microcalcification
at 2-3 o'clock measuring (5.5*3 cm) 1.2 cm ANM NAC

Mets Workup :

CT CAP -free

BX
IDC

ER
0/8

PR
0/8

Her 2 Neu
3

SISH

Ki67
50

T
3

N
1

M
0

Sonomammography

Cesm: UOQ mass+ satellite

PATIENT HISTORY

received NACT TCHP x 18 wks with Hp X 5 last 1/2/2025

cesm:lt breast BIRADS 6 status post therapeutic show no pathological uptake related
to the inserted clip with surrounded highly suspicious linear calcifications

CESM NO UPTAKE

LT MRM :free nipple , areola, LN 0/16 , deep margin

complete pathological response

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR HERCEPTIN S.C UP TO 17 CYCLE
PORT

SIGNATURE

MEDICAL ONCOLOGY

Signature of Medical Oncologist

RADIOLOGY

Signature of Radiologist

SURGERY

Signature of Surgeon

PATHOLOGY

Signature of Pathologist

Psychiatric

Signature of Psychiatrist

Nutrition

Signature of Nutritionist

Name: زينب محمد امين محمد		
MR. : 5341		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient was scheduled for adjuvant therapy with HERCEPTIN 600 mg subcutaneously, with a planned echocardiogram for baseline cardiac assessment prior to initiating systemic chemotherapy. The patient was diagnosed with left-sided breast cancer, clinical stage T3 N1, estrogen-receptor negative (ER 0/8), progesterone-receptor negative (PR 0/8), HER2-positive (HER2 23+). The treatment plan included neoadjuvant chemotherapy (TCHP) to begin on 15 March 2025, followed by left mastectomy with sentinel node biopsy (MRM) and consideration of adjuvant radiotherapy (CR N0/16). New laboratory studies and a repeat echocardiogram were ordered to assess suitability for HERCEPTIN subcutaneous administration. The patient completed the TCHP regimen and underwent a sonomammography on 24 February 2026, which was reported as negative. The implanted port was removed without complication. A chest X-ray performed on 2 March 2026 was reviewed and found to be unremarkable; no evidence of metastatic disease was noted. The patient remains under observation. On 7 July 2026 the patient presented for follow-up; performance status was ECOG 1. Ongoing follow-up since 13 March 2024 continues without new findings. A chest X-ray performed in April 2026 was negative for metastasis. The patient reported postoperative muscle pain, for which local Voltaren gel and massage were recommended, with a repeat visit scheduled in two months.		
تحريراً في ,		
	التاريخ :	19-09-2026
	الوقت :	
	Reham Ahmed Fathy Ibrahim	الطبيب المعالج :
يعتمد ,		

Presidential Initiative for Women's Health
Pathology Lab - Cairo University Hospitals

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي
مستشفيات جامعة القاهرة



PATHOLOGY REPORT

Name: زينب محمد امين محمد
Patient's I.D : 27302142100582
Age: 40

Pathology Number: 129/2014
Specimen Received Date: 21/05/2014
Date Report Issued: 07/05/2014

Clinical data: Left breast mass.

Nature of the specimen: U.S. Guided Trucut Biopsy

GROSS EXAMINATION: Tissue core biopsy, totally submitted.

MICROSCOPIC EXAMINATION: Serial sections examined revealed cores of breast tissue containing malignant tumor formed of tubules and groups of moderately anaplastic epithelial cells within desmoplastic stroma with 5% ILS.

IMMUNOHISTOCHEMISTRY PROCEDURE: Sections from paraffin block were treated with antigen retrieval solution and then stained with monoclonal antibodies for estrogen receptor (ER) (SP1) Rabbit anti-Progesterone Receptor (PR) (1E2), CONFIRMTM anti-HER 2/neu (C-11) Primary Antibody, and CONFIRMTM anti-Ki67 (30-9) Primary Antibody, using Ventana ViewTM system. DAB was used as a chromogen and hematoxylin as a counter stain.

IMMUNOHISTOCHEMISTRY RESULTS:

- Staining of section treated for estrogen receptors revealed negative nuclear staining in tumor cells by chromogen.
- Staining of section treated for progesterone receptors revealed negative nuclear staining of tumor cells by chromogen.
- Staining of section treated for Her-2/neu revealed strong complete positive nuclear staining in >10% of tumor cells by chromogen (score 3+).
- Staining of section treated for Ki67 revealed positive nuclear stain in average of 50-60% tumor cells.

DIAGNOSIS: Breast Mass, U.S. Guided Trucut Biopsy

- Invasive Duct Carcinoma, NST, Grade II.
- Negative For Estrogen Receptors, Score 0/8.
- Negative For Progesterone Receptors, Score 0/8.
- Positive For Her-2/neu Expression, Score 3+.
- Ki67 Proliferative Index.

02
018
018
3+

DR. PASSANT ESSAM
DR. AYMA MAGDY

Patient Name :	المرضى / د. منى محمد محمد	Request No. :	13009875
Patient No. :	27302142100582	Request Date :	15/03/2025
Path No. :	1035/25	Verification Date :	09/04/2025
Sex :	Female	Financial Category :	أورام بنى السلام - فئة الدولة
Age :	52 Y	Department :	Out Patient
Referring Doc :		Registered by :	marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:

- * Procedure: Left MRM measured 12x10 cm with covering skin ellipse measured 10x8 cm.
- * Tumor extent: An ill defined grayish white firm tumor bed site.
- * Size: 7 x6 cm.
- * Skin involvement: Nipple & areola were unremarkable grossly.
- * Distance from margins: 1 cm away from deep margin.
- * Lymph nodes number and size: 17 brofatty tissue piece measured 8x7 cm, entangling 16 L.Ns up to 1.5x1 cm.

Microscopic Examination:

- * Histological type: Minimal residual DCIS. No remanants of invasive tumour.
- * Skin involvement: Free nipple and areola.
- * Lymph node involvement: Negative (0/16)
- * Resection Margins: Free deep margin.
- * Histologic response to therapy: Complete pathological response
- * Microcalcifications: No.
- * Additional pathologic findings: No

Diagnosis:

- * Left breast cancer post NACT, MRM specimen:
- * Minimal residual DCIS. No remanants of invasive tumour.
- * Free nipple and areola.
- * Lymph nodes were negative (0/16).
- * Free deep margin.

Verified by

DR Menna Halsei

Printing Date : 13/04/2025 11:04:55

Page 1 of 1

Printing By : marwa mohamed youss

Request No.	: 13009875	الإشعار رقم	: 13009875
Request Date	: 15/03/2025	تاريخ الطلب	: 15/03/2025
Verification Date	: 09/04/2025	تاريخ التحقق	: 09/04/2025
Financial Category	: Out Patient	التصنيف المالي	: Out Patient
Department	: Out Patient	القسم	: Out Patient
Registered by	: marwa mohamed youss	سجلت بواسطة	: marwa mohamed youss
Referring Doc	:	أرسل الطبيب	:

Pathology Report

* Complete pathological response to therapy.
- Stage: ypTis, ypN0.

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil

Verified by
DR Menna Nabil

Printing Date: 11.04.2025 11:01:55

Page 1 of 2

Printing By: marwa mohamed youss

مركز راد (4) - الدور الأول - مبنى راد (987) - فورتلبيس النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mena El Qadima - Cairo, Egypt



Patient Name: Zainab Mohamed Ameen Mohamed	Date : 08/02/2025
Exam : MAMMOGRAPHY BOTH	Age : 52 Year(s)
Medical ID: 1699/2024	Pat. ID: 78822

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Left breast cancer, on neoadjuvant chemotherapy

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- Left UOQ radioopaque clip surrounded with malignant looking microcalcifications 4.9x2.8x2.7 cm seen 1.5 cm from the nipple, with no underlying findings seen by US
- Bilateral scattered benign looking microcalcifications
- No suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound showed no right solid or cystic mass lesions.
- Bilateral retroareolar dilated ducts with few showing internal echogenic avascular contents
- Normal skin thickness and nipple-areola complex.
- Right non-specific axillary LNs.
- Left multiple prominent axillary lymph node showing diffuse cortical thickening and preserved hilum seen on level I,II

Opinion:

- Left UOQ partial response of malignant mass (BIRADS 6)
- Bilateral retroareolar duct ectasia with inspissated secretions (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada

08/02/2025 12:45 PM

[Handwritten signature]



Patient name english: ZENB MOHAMED AMEN	Age:
Study Date: 5/12/2024 11:35:42 AM	Patient id: 24036704

- No pathological abdominal or pelvic lymph node enlargement.
- Bilateral pelvic phleboli.
- Retroverted uterus noted.
- No ascites.

OPINION:

- Hepatomegaly with segment II focal lesion keeping with hemangioma.
- Two small splenic focal lesions with enhancement pattern keeping with hemangioma.

Dr. Eman Abdeen, MSc.

Dr.Noha AlMansy .MD



Patient name: ZENB MOHAMED AMEN	Age:
Modality Name: CT	Study Date: 5/12/2024 11:35:42 AM
Patient id: 24036704	

THORACIC IMAGING UNIT

POST CONTRAST CT SCAN OF THE CHEST REVEALED:-

**** Reported history of patient with left breast carcinoma presented for metastatic workup.**

- Fairly defined left breast heterogeneous soft tissue mass lesion is seen associated with prominent left axillary lymph nodes at level I & II (largest measuring about 1.8 x 1 cm in maximum axial diameters). Associated few small left internal mammary lymph nodes are seen with largest one measuring about 0.6 cm in maximum diameter.
- No evidence of consolidations, cavitary lesions, bronchiectatic changes or pulmonary masses.
- No sizable enlarged mediastinal or hilar lymph nodes.
- No pleural or pericardial sac collections.
- No gross cardiomegaly.

OPINION:-

- No gross pulmonary metastatic deposits.

Dr. Marianne Abdallah
Dr. Mohamed Raafat, MD



Zeinab mohammad Amin

Female

Q42570

9/30/2024 11:30:48 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of left breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- No abnormal osseous tracer uptake is seen that would account for bone deposits.
- Degenerative changes of the spine are noted.
- Osteoarthritic changes of the both knees and shoulders are seen.

Opinion

- No scintigraphic evidence of metastatic osseous lesions.

Dr. Moh amed AlaaEldin

Prof. Ahmed Kandeel, MD